

EDITORIAL

How Much Should Hepatitis C Treatment Cost?

By The Editorial Board

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A new pill to treat hepatitis C raises difficult questions about fair pricing, not only in the United States and other affluent nations but in developing countries around the world. Hepatitis C, which afflicts some 150 million people globally, often without symptoms for years, can cause fatigue and fever, cirrhosis or liver cancer.

The pill, known as Sovaldi, or generically as sofosbuvir, costs \$84,000 for a standard 12-week course of treatment. That breaks down to \$1,000 for each pill, taken daily. The manufacturer, California-based Gilead Sciences, says private insurers are already covering Sovaldi. The question is whether insurers and public programs like Medicare and Medicaid should have to pay so much for this drug, driving up costs for taxpayers and private policyholders.

Gilead says the price is consistent with the cost of previous treatment regimens (a contention disputed by independent experts) and is reasonable given that the drug can have fewer side effects and cures a higher percentage of patients compared with other drugs. In the long run, the company claims, Sovaldi could save money for insurers by preventing patients from getting sicker and needing costly medical care.

However, a panel of experts at a recent forum in San Francisco concluded that the drug offered “low value” for treating most patients, in large part because of its high price. That judgment was based partly on an assessment of clinical and cost-effectiveness prepared by a nonprofit organization that evaluates medical treatments (the Boston-based Institute for Clinical and Economic Review). The group estimated that replacing current care of infected Californians with Sovaldi-based regimens would raise drug expenditures in the state by \$18 billion or more in a single year. It said projected savings from reduced medical costs in later years would not come close to offsetting that cost.

The drug will be sold for much less in many other countries. Gilead plans to license three to five Indian drug makers to produce Sovaldi and expects the price to be perhaps \$2,000 for six months of treatment through public hospitals and community clinics in India (compared

with \$168,000 for six months in the United States). Gilead said similar arrangements might be made in 60 low- and middle-income countries. Prices in Europe are high, but not as high as in the United States.

What can or should be done to reduce the cost in the United States? Some experts suggest providing the drug only to patients with advanced liver disease. Others hope that other new drugs in late stages of clinical testing will be approved, adding competition that could help restrain prices. Still others urge that state Medicaid agencies, which cover a big portion of the infected people, negotiate with Gilead for lower prices or find ways to limit the drug's use when other good options are available. There is no easy way to contain costs as expensive new drugs come along, so all of these strategies will need to be tried.

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